

Aisha Patel

MR NUMBER	MR-38201	VISIT DATE	Dec 10, 2024 · 10:55
PHYSICIAN	Dr. Asif Imam	VISIT TYPE	Comprehensive Note

BP 128/84 mmHg

PULSE 92 bpm

TEMP 38.5 °C

SPO₂ 97 %

WEIGHT 72 kg

CHIEF COMPLAINT

Productive cough

Fever

Sore throat

HISTORY OF PRESENT ILLNESS

55-year-old male presents with a 4-day history of productive cough with yellowish sputum, fever peaking at 38.7 °C, and mild sore throat. Onset was gradual. No known sick contacts. Patient reports poor appetite and generalised fatigue. No chest pain or shortness of breath at rest. Tried OTC paracetamol with partial relief.

ALLERGIES

Penicillin

Anaphylaxis / generalised rash

Severe

NSAIDs

Gastrointestinal upset

Moderate

MEDICAL HISTORY

Hypertension — diagnosed 2019, on Amlodipine
Seasonal allergic rhinitis

SURGICAL HISTORY

Appendectomy — 2015 (laparoscopic)
Tonsillectomy — childhood

FAMILY HISTORY

Father — essential hypertension, stroke at 68
Mother — Type 2 diabetes mellitus

SOCIAL HISTORY

Non-smoker

Occasional alcohol (social)

Office worker — sedentary role

Married, 2 children

REVIEW OF SYSTEMS

Cough — productive, yellowish sputum
Fever — present (38.5 °C)
Sore throat — mild
Shortness of breath — absent
Chest pain — absent
Nausea — mild

PHYSICAL EXAMINATION

General: Ill-appearing, febrile, not in severe distress
ENT: Mild pharyngeal erythema, no exudate, no cervical lymphadenopathy

Chest: Bilateral coarse crackles at both bases on auscultation
CVS: Regular rate and rhythm, no murmur, no added sounds

POINT OF CARE LABS

Rapid Strep Test	Negative	Normal
CRP	48 mg/L	Abnormal
Finger-prick Glucose	98 mg/dL	Normal

ASSESSMENT & DIAGNOSIS

J06.9	Acute Upper Respiratory Infection	Moderate
J20.9	Acute Bronchitis, unspecified	Low

LAB ORDERS

CBC with Differential	CRP / ESR	Throat swab C&S
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PRESCRIPTIONS

DRUG	INSTRUCTIONS	QTY
Azithromycin 500 mg	Once daily × 5 days	5
Paracetamol 500 mg	TID × 3 days PRN fever	9
Salbutamol Inhaler 100 mcg	2 puffs TID	1

PROCEDURE ORDERS

Nebulization with Salbutamol 2.5 mg — stat dose

CARE PLAN

Steam inhalation TID for 3 days
Oral fluid intake ≥ 2 L/day
Complete rest — avoid exertion for 48 h
Return if fever > 48 h or dyspnoea develops

REFERRALS

ENT	Persistent sore throat not responding to empiric therapy
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PATIENT GOALS

Complete full antibiotic course without missing doses
Maintain oral hydration ≥ 2 L/day
Return immediately if shortness of breath or worsening fever

OTHER ORDERS

Medical rest note — 3 days (employer certificate provided)
Isolate from immunocompromised household members

VISIT SUMMARY

Acute respiratory illness with probable bacterial component. Initiated antibiotic therapy, symptomatic management, and nebulization. ENT referral placed. Patient educated on infection control and return precautions.

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FOLLOW-UP

14 Dec 2024

Attending Physician	Visit Type	Facility
Dr. Asif Imam	Comprehensive Note	NovaDoc Health